TCF induction followed by radiotherapy

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

After induction · radiotherapy alone under the separate plan

Active medicines:

Emetogenic risk: Minimal

Source-linked clinician/pharmacy draft. Confirm active phase, doses, routes, steroid totals and start/stop dates; no administration authority.

Additional phase-specific support

- If there is no progressive disease and marrow function is adequate, radiotherapy starts within 4–7 weeks after completing chemotherapy; surgery is considered afterwards. This source describes radiotherapy, not routine concurrent carboplatin.
- No routine concurrent carboplatin or induction systemic anticancer therapy (SACT) support belongs to this source-defined radiotherapy (RT)-alone phase.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00324 v6b · pNCCP 00324: treatment/supportive-care sections; NCIS v8 pp4–5 · NCCP 00324 with current NCIS and local supportive-care policy

Sources checked 2026-11-10 · SACTCheck v0.84.48 · Regimen-level drafts; prescriber and oncology-pharmacy verification required